

Mon, Jan 09, 2023 5:34AM • 45:23

**Therapist** 00:00

Got it?

**Interviewer** 00:04

All right, I will not be keeping the video, it will just be for a transcription purposes.

**Therapist** 00:10

Okay.

**Interviewer** 00:12

All right. And just start this as well. I wanted to get a better sense of your clinical practice. What is the primary modality that you do for couples therapy?

**Therapist** 00:24

So well, I have a training in solution focused brief therapy. So here in the [Country A], the training takes around [1-5] years. So I did. I have this accredited training that took me [1-5] years, and I finished it, I think, last year, but I was working with couples already during the training. And I'm also a psychologist. So I've studied psychology, I have a bachelor degree in psychology and a master's degree in psychology. So I do this, I guess my approach is mainly mainly stems from the solution focused brief therapy. So that's, that's part of the systemic approaches or systemic approach in psychotherapy. But I'm also inspired by other techniques and approaches like EFT or the work done by what is the guy's name? The book is called couples therapy, the non non traditional approach. That's a book by Dan vile. So I was also inspired by this book when I started to work with couples. So I guess it's kind of eclectic a bit, but mainly stems from the solution focused therapy.

**Interviewer** 01:49

Interesting. And do you provide your service as a specific type of plans? Are there specific couples that provide your services?

**Therapist** 01:58

Well, I started in a in a state counseling institution here in the [Country A], we have site centers in every region. So it's a social service. So every couple can go there. And it's for free. And so there are all sorts of different problems. But mainly, all the centers are for families, for family therapy, and for couples therapy, but mainly, mainly couples come for the treatment. And I was there for [1-5 years]. And now I have a private practice. I'm calling you from the private practice. So the covers are usually there on the on the couch. I'm sitting here. I wouldn't say I have a, like a specialty when it comes to working with couples. So they come with all sorts of problems, mainly, communication, intimacy, conflicts related to upbringing of children. So different topics.

**Interviewer** 03:10

And you've always practiced couples therapy in [Country A]?

**Therapist 03:17**

Well, I do both. Obviously, I would say I, I, I had more couples in the past. But as it's quite a demanding job, now I have a little less so now I would say I have like 50% couples 50% individual therapy. I used to have slightly more I used to have like 60 to 70% couples therapy and the rest is individual therapy.

**Interviewer 03:43**

Are all of your couples based within [Country A]?

**Therapist 03:48**

Most of them, yeah. Sometimes one of them or the couple goes abroad. So I continue with an online therapy. But as I speak English, then I also work a lot with expats or with couples where one of them is a foreigner and one is [Nationality A]. By usually they are based in [City A], I'm in the city center of [City A]. So it's very often that they leave for the work nearby the city center, and that's also why they want to come here.

**Interviewer 04:21**

Interesting. All right. Actually, I was going to ask you as well that when clients come to you, I wanted to ask what usually is the presenting issue, but you said that it was communication, intimacy, and for example, problems in bringing their child is that usual? Is that usually what they seek your service for?

**Therapist 04:43**

Yeah, I would say mostly they start like the topic is communication. But obviously there's there's so many topics behind this, you know, it might be related to values it might be related also to the conflicts related to the outbreak. Being of the children that they have a different approach. Sometimes it's intimacy, usually related to frequency that one of them wants to have sex more and the other one, not so often. It's not really related to, I would say, like sexual techniques that one of them wants it, like BDSM. And the other ones, regular sex, it's not really often it's more about the frequency, I would say. They decide to look for counseling, and what else? Sometimes they come from a court that when they want to divorce, then the court says, well, we don't really want to divorce you first try a family therapy or a couple of therapy. But usually, it's not very effective, because the court usually says this after many years of the conflict of the of the court procedure, and then the, you know, the couple is really fucked up at this point so,. It's not really very efficient. And this is this is it actually is not? Well, sometimes the court says you need therapy, but when they come, it's more often a mediation where we try to settle the agreement about division of the childcare, whether it's exclusive care, shared parenting, or whatever, because here in the [Country A], they have to come with an agreement to the court, in order for the court to be able to divorce them, they have to have the agreement first. So I help them with this.

**Interviewer 06:37**

That's interesting. And do you do a formal assessment when clients come into your practice?

**Therapist 06:45**

Well, it's not really a formal assessment, what I did in the, in the, in the state counseling institution where I worked, we had a sort of an enemistic review, where we asked a couple of information about the clients and I do the same. So I asked them their name, I asked what they studied, I asked, what is their occupation? What do they do for the living? I asked for their age? Then I asked whether they have children? How old are the children? And what is their name? Then I asked, How long is the relationship? And how long is the marriage if they are married? So I, I asked about these things. Yeah. And then I explained them a little bit in the beginning, how is the therapy? How is it going to work? The cooperation with me that they can cancel, you know, 40 hours before and that I'm confidential about everything they tell me. So all these, you know, standard things that I guess we, we all say at the beginning?

**Interviewer** 07:54

All right. So you've never a use of pen and paper, like relationships scales etc.

**Therapist** 08:04

Not really not a relationship scale, what I'm using. I don't do it. Like, I wouldn't say I do it every time. But I needed it for my training in psychotherapy. And then with some of the clients, I still stick to this to, I guess, you know, it's an outcome rating scale. Yeah. So I don't know if I have it here, outcome rating scale by Scott Miller. So this is what you gave at the beginning of the session, to see what is their level and then then on the next sessions, you again, give it the very beginning of the session to see if there is progress. And then you can discuss with the partners, that for one of them, maybe there is a progress, there is not the progress for the other one. So you can start or I can start the session with them. I usually do it for the first session, and maybe for the couple of the first sessions, and then when the cooperation is about to finish, then I give it to them once or twice to see that the really the numbers are better, and then I can I can see that they they are free to go.

**Interviewer** 09:13

Actually, I'm not that familiar with a scale, I can really scale what would be in it.

**Therapist** 09:20

I can I can well, I can write the name it's how it's called. And yeah, let's see one of the authors and we have a translated version. And you have to actually measure it. The numbers and there is a cutting of score which is 25. And if they score above 25, then score, assess and they have done some research on that some psychometric research. I believe that when the client is Above 25, that is no longer in need of treatment of psychotherapy. So it's like, I guess it's the only only thing I'm using at the moment. And but I want to use one more. And but I'm still still didn't do enough effort. thing. Let me find it for you. It's the look that in one of the [University A]. And it's like a software it's called the deepPsy, like psychology. It's a [Country A] thing. I'm sending you the link. It's like a new web app. And I think it's also based on the outcome rating scale, somehow this is involved. So this also help you measure the outcome of the

**Interviewer** 11:00

Interesting, do you pay for the skills? The ORS?

**Therapist 11:10**

No. Outcome rating scale is for free. And the deep Psy? I think at the moment, they got some sort of a grant funding, so you don't have to pay for it. But I think they want to make it as well, you will have to pay for it in the end, but maybe not at the moment, I'm not sure.

**Interviewer 11:31**

And you have any challenges when you use for example, the ORS?

**Therapist 11:35**

Sorry?

**Interviewer 11:37**

Do you face any challenges or difficulties when you use a scale, the outcome rating scale?

**Therapist 11:43**

Well, the thing I don't like about the ORS is that I have to administrate it via paper and a pen, and then I have to measure it, it's sort of so the reason why I want to switch to deepPsy is also that I will have it on an iPad on a tablet, and I will give it to the clients on the tablet. But for this I have to buy the iPad. So it's kind of postponing, postponing the moment?

**Interviewer 12:17**

And how informative do you find ORS?

**Therapist 12:22**

Well, it's like, second opinion for me, because I usually do like a feedback part. By the way, of course, at the beginning of the session, when I asked the people what has improved if there was any change, etc. And then I also asked the clients in the end of the session about the session, how was the session and what they're taking away. And sometimes when the clients are telling me that things are getting better for them, then then it's also good for me to see the numbers if there really is a change, when compared to when they came for the first time. So it's kind of like did I really have like another opinion or another source of information that they are really doing fine.

**Interviewer 13:10**

That makes sense. All right. And the email as well, I think I touched upon this, that across the years, different therapists and different researchers have characterized the construct of relationship quality quite differently from each other. I wanted to get your view about for you, what is relationship quality?

**Therapist 13:33**

When I got your email, it actually got me thinking because I don't really, I don't really have a concept of what the good, like what a quality relationship is, because I usually leave it up to the couple. So like, my understanding is that when they are fine, they are not sitting here they don't meet me, and when they are not fine. And then it's really up to them to explain me what is not fun with the relationship. So, so

first, I have to, I have to get the information from them. And it's also interesting to see whether they think if it's a shared problem, whether they think that intimacy is a problem for both of them, or whether they have a different opinion on that or whether they think that there's something wrong with the relationship, but each one of them thinks a different thing. So I don't really like have a concept of a good relationship, I would say I leave it up to them. So they have to tell me what what they would like to you know, change or improve or achieve in a relationship so that they feel better there.

**Interviewer 14:47**

That's interesting, all right, because maybe all like a different way to ask it or maybe like a different tab. It would be to ask for you like what differentiates a couple with higher Relationship Quality, were the couple that has low relationship quality?

**Therapist 15:09**

Okay, so I think there has to be a satisfactory level of, of intimacy and sexual encounters. And this really depends upon the couple, it might be once in five years or three times a day, you know, so, but usually, it's some something in between once the research says it's once per seven or 10 days for the for the long term couples. So, there has to be this then for the high scoring satisfaction, I guess there is like, like a good communication basis, there is a mutual agreement on the important things in the relationship, like for instance, finance, that they agree on how they are going to spend the money, and on the board and how much because this, this could be also one of the sources of problems in the relationship, then they have mutual agreement on how they are going to treat their kids, if they have kids. And they have also that there is like way, a good way a satisfactory way for them to spend a free time. So that there is if they have no kids if they have a free time that they can enjoy the time together. Without, you know, just to enjoy the time together, and it it it can be can be a hobby, it can be traveling, it can be whatever watching Netflix, I don't know, if they enjoy it together, it's fine. Now where I am, I said it's the it's the sexual intimacy, it's the communication. It's the agreement on finance and upbringing of the kids. And it's the enjoyable spending of the of the free time.

**Interviewer 17:26**

All right, so would these be your question, the next question of mine would be like, in your professional experience, what are the most important things in a high quality relationship? So would you say that these are the things that are the most important?

**Therapist 17:48**

Well, I would say that the most important thing is that they, they need to have a way to communicate their differences, you know, they do not have to agree with one another, but they have to have a way to to have the empathy from the other person to gain the empathy and also to be able to give empathy to the other person. And when they do not have this, well, sometimes they can experience in the therapy through me or through the sessions. But when they do not have this, then it leads to alienation of the couple. And then with the alienation comes devaluation of the person that then lack of sexual encounters. All other conflicts. So for me, I would say the most important thing is like to have a space to have the ability to, to talk to one another in a way that is productive. It doesn't, it doesn't necessarily have to be a peaceful way, it can even be a conflict because sometimes the couples, some couples

need to have a conflict in order to overcome their problems. So they need to have a way to talk about their global problems and make it productive.

**Interviewer 19:08**

Interesting. And earlier, you said that, you know, if you don't have a concept of like relationship quality, because you're it's really leave it up to the top. Right. Yeah. But I wanted to ask you about that as well. Because, look, that's how you personally or professionally, think about relationships. That's how you conceptualize relationships often differ from how your clients conceptualize relationships. And does this impact that therapy?

**Therapist 19:44**

Well, yeah, I think that's impact therapy because you sometimes in the therapy, it's interesting to ask the couple and the partners about their expectation towards the partner, maybe they have a model in their old family, how the man and the woman or are men and the men, I don't know how they should be. And it's different in the new relationship. So sometimes in important, it's important to say it out loud, to see how this influences their behavior and the relationship they are in. But this is I guess, like, this is more important for me when I work with the couple, then would I think a proper relationship should be Yeah. And it but I guess it stems from it from my, my training and psychotherapy, because solution focused therapy is, is an approach without theory of psychopathology there, it's like, they don't, they don't really try to understand what the problem is, they want to come up with a solution. And that's why it's a solution focused therapy. On the other hand, I'm a psychologist, so I, you know, read many theories about relationships, I don't, I don't find it useful to impose this theory on the couple that is sitting in front of me and to tell them look, a man should get a red car, and a woman should stay at home. And that's how it should be, you are so wrong, don't do this. You don't do this. Every therapist, I mean, a therapist wouldn't be very successful.

**Interviewer 21:37**

I wanted to ask you, as you know that the relationship is a complex system, it involves love, sex, identity, relationship, satisfaction, conflict, all of that journey therapy, how do you decide which one to focus on, or which one of this complex system to target?

**Therapist 22:00**

Well, it also depends on the duration of the therapy. So if it's a shorter one, or if I see them for the first time, or just two or three times, then I really focus on the most important matter. Yeah, so usually, it's like the thing that that brought them to therapy. So it can be as I mentioned, money or communication, or finance or whatever. But when the when the duration and the cooperation is longer, then obviously we can touch a lot more topics, like all the topics that you mentioned, we can touch them, one after another. And and I usually try to do sort of a short contract at the beginning of each session. So I try to make an agreement on what we are going to go through this particular session. So then again, it's up to the couples to tell me what they want to talk about today. And then I kind of follow them there.

**Interviewer 23:14**

All right And when then do you determine when to end couples therapy,?

**Therapist 23:19**

when to ended up? So when the person or when the couple is, is coming for like two or three times and I see that they are doing fine. Then I usually open the topic of ending the therapy. And I asked them about it. That's also the point when I do the scoring through the outcome rating scale to see if they have really improved. And then, but then again, I leave it up to them to decide. So I want to hear their opinion. Sometimes they say yeah, it's it's doing fine. So maybe we can do one more. Sometimes we do an agreement that we do one more meeting in about three months to see whether they are still doing fine. Sometimes I leave the option to have the meeting in three months, but they have the option to cancel it like two weeks before it's coming so they can just write me a message we are doing fine. We don't need this checkup meeting.

**Interviewer 24:38**

So is that usually the case that two to three sessions is already good for for many of the couples that you see.

**Therapist 24:45**

Now it's not but sometimes sometimes it is and it's very satisfactory for me. So sometimes we can we can overcome the problems in let's say in a very short time in like two or three sessions, as I said, but more often is that we do like, I don't know, six to 10 sessions. And then then it's sort of improved, they take three months of the break, and then they call me again, it's terrible. Again, we have to meet you again. And then we do another, I don't know, 2, 3, 5 sessions. So for many of the couples, I'm like this, this person to be called when, when they have conflict, and I see them, I see them for a couple of years on an ongoing basis.

**Interviewer 25:43**

Interesting, alright. And how often are these sessions usually, like, once every week, once every two weeks?

**Therapist 25:51**

It's usually once per three weeks, when we start once per two or three weeks when we start, then for the ongoing cooperation, it's once per three weeks. And then when they are fine, and we are trying to finish the therapy, then I can see them for like once per month or two. And then for the final one in three months, and then it's over. So I'm making actually making the duration the periods longer.

**Interviewer 26:26**

Interesting. And how long are the service therapy sessions?

**Therapist 26:32**

60 minutes?

**Interviewer 26:34**

60 minutes

**Therapist 26:37**

Yeah, I don't want to see them longer. I mean, some people, some people do 90 minutes, but I didn't find it useful to have 90 minutes. But sometimes, you know, people come during the day, they have to go back to their work. And when the when the process is difficult. It's better if the process is difficult for 60 minutes, then for 90 minutes.

**Interviewer** 27:00

Interesting.

**Therapist** 27:02

So it's also for them, I think it's better that it doesn't take one hour and a half.

**Interviewer** 27:11

I wanted to ask as well. And I touched upon this in the email that we are currently developing a relationship quality scale that hopefully better represents the relationship as a complex system that would help therapists and treat both the treatment planning and the treatment evaluation. How interested would you be in this kind of scale?

**Therapist** 27:33

How interested? Well, I think I would be interested in using it. I'm wondering about the norms and the psychometric qualities of such scale, because I guess there would be norms for the country of origin of the scale, maybe [Country B] and you know, but you know, then then there is question whether it would work for for the [Country A]. If it's without the norms, then then for me, it's interesting to see just the progress. Yeah, that's, I find it also helpful.

**Interviewer** 28:14

All right, we are planning to do it in Brazil, the Philippines, Netherlands and France. So which will be the data gathering countries and maybe the US? So we're planning to have norms and validation from those four countries? Five countries sounds great. Presently, what would you be looking for in a newly developed scale like this? How can we make sure that a scale like this case, it would be useful in your practice as ever?

**Therapist** 28:43

Well, I would have to translate it in the first place. So a lot easier would be to already have it translated by someone else. Otherwise, I could use it just with with English speaking clients, which I have some but the but not everybody is. But the best way for me is to use it like with, with everybody not to have like one thing that I use with everybody. And I make it a habit with all the clients. So there could be the language barrier. Also, the problem is when it's when it's a web tool or software, then I would need two iPads. Yeah, otherwise, it takes a lot longer to give it to like first one of them. The other one of them, then it will take like, I don't know, we'll take like six, seven minutes, which from the 60 minutes is already quite some time. So I would be I would like to have a short to maximum, four or five scales, something like that. Not really

**Interviewer** 30:01



Do you do give? For example, would it be like would it be able to integrate in your practice? If it were a longer tool? Would you be able to give it to them? Not in session? Like before they come in? Answer, yeah, then you would have the results.

**Therapist 30:16**

Yeah, that's a good idea, I could do that too. So I could, I could send it to them before the session via email, and I could read it or get to it at the beginning of the first session, for instance, and then I have a shorter two, maybe four, for an ongoing basis for them that I could give already in administer already, at the beginning of the second and another session.

**[Portion Redacted]**

**Interviewer 33:36**

no problem at all. But that's useful information to have. Thanks so much. I also wanted to ask, of course, you already, you know, contributing so much just reading into it here. But I also want to ask, Would you be interested in further collaborating on the development of the scale?

**Therapist 33:54**

In what sense would do I do

**Interviewer 33:58**

If ever it would be, once we develop the scale, it would be sort of feedback from you, either. You know, like how the scale? What do you think about the scale and cetera, et cetera, just sort of feedback or either. For example, once we're done with the interviews that we're going to be interviewing relationship therapist, couples as well. And we're probably going to be identifying certain domains, we'll be asking our network to create items, then you create a final version of the scale. So it might be the case of either just reviewing from like, what you think how it looks like, etc. Or it might be the case also that we will give you the scale once it's finished, and see how you use it in your practice. Like firstly, like, for example, like we'll give it to you for a period of six months and say, after that six months, like how did you feel about that? How do you what do you like about, etc.

**Therapist 34:57**

That's interesting. Yeah, I would Be willing to give feedback, and maybe even using it using the prototype

**Interviewer 35:08**

Alright, that's thank you so much. All right. Now, actually, we're just wrapping up, I just like to go over very quickly about what you discussed so far in this session. At first, I wanted to get a better sense of your practice. So you say that primarily, it is a solution focused brief therapy, which is a systematic approach. But but you're also inspired by EFT. You're also inspired by the couples therapy or non traditional approach. But then while for you, it feels eclectic . But again, primarily on the solution focused approach. You used to be working in a state counseling, now you're working in private, you don't specialize in any certain type of couples, but it's more general more complex. Most of them are in [Country A], but also by the bed, some, some are like one [Country A], one is abroad, or one is the next

fast etc. So when clients come to you, you say that the usual presenting issue is about communication. But that communication issue is underlying values, upbringing of children, intimacy, frequency of sex, etc. You also said about the court cases where before divorce, they need to see you, which usually just ended up as mediation for for the division, for example of child care. Usually, it's a harder case as to resolve because all the years of conflict, as you said, you said that you don't do your pen and paper assessment, what you do is more like an intake interview, as a name, study, occupation, etc. And what they need from therapy are their treatment goals. But you do use the outcome rating scale by Scott Miller. And you do want to use deep sigh, which you administer, you don't systematically administer it, but you do sometimes give it in the first session, and then on the sessions afterwards. What you don't like about it is that you administrate the paper. Also, you also do like the calculations, we have the upper, like manual calculations, as well as their results in ORS. Is that great?

**Therapist 37:22**

Sorry, what was the last thing? Yeah,

**Interviewer 38:24**

Because the administrator is on papers, so you need to like calculate it on your own as well, then we'll cetera?

**Therapist 37:29**

Well, you will need to measure it with a ruler. Oh, really? Yeah. Because the scale is like, there are no numbers on the scale. It's just a scale with. Like, there's, for instance, the scale is called a relationship and you put a mark. But, but this scale is, like 10 centimeters. So then you have to measure it, and you calculate the point. So like, 7.2. And then you have to count it all together, I have an Excel sheet for that, where where the final overall score is calculated. But you know, measuring it with a ruler, it's like a 19th century thing.

**Interviewer 38:15**

That's, that's funny. Oh, yeah. So I also asked you about relationship quality, and for you don't have a concept for it, you leave it up to the couple, you know, if they're fine, they're not gonna be there, if they're not fine. Again, it's been awhile. But then they also asked you in a different way, like what differentiates higher relationships from low relationship quality couples. And then you mentioned four things if, you know, a satisfactory level of intimacy of sexual encounter, it is of good communication, and mutual agreement on important things like kids like, on treating their kids, etc. And also like a satisfactory way of spending free time, whether it's traveling, whether it's Netflix, et cetera. But for you really are really important thing for a relationship is communication, having empathy, you know, productive ways to talk to each other, even if that productive way is conflict that you said that some people need contact? Otherwise, it would lead to alienation.

**Therapist 39:18**

Yeah, maybe the last point is now when I think about it, the last one is related to, to secure attachment, what I meant by having the space to communicate with one another than I meant to have like secured attachment that both of them feel safe in the relationship.

**Interviewer 39:43**

All right. And then we I asked you about, you know, how to conceptualize their relationship and how you conceptualize it and does that impact therapy? For you? It's really like explicating how they learned about relationships, for example, through attachment figures that you want to work to carry out, you know, which is important for your work and therapy. And then I asked like, how do you how do you focus on which of the complex system in the relationships? And for you, it depends on the duration, if it's only two to three sessions, then you know, go to the most important thing, the one that they said first otherwise, but if it's longer than you can get to the many other things that are in their relationship. So usually for you, what is it? Is this a state? Sorry? Is this a state? Kind of thing? That's two to three sessions? What is there like a reason for for this number? Or is it usually?

**Therapist 40:50**

Well, I would say it's, it's either successful with a couple surprisingly successful with one or two sessions, and then we can really finish, like after the third session, or I'm unsuccessful with the couple in the first one or two sessions, and they can see then they decide not to continue with me. Outside, it's both it's both cases. Okay, why it's really difficult to tell because bringing new couple can be anything it can be like, one session thing, it can be two sessions it can be we are seeing for 30 other sessions. So it's difficult to to answer this question, what is what is the typical duration of my client?

**Interviewer 41:48**

All right, I was just wondering if that in [Country A] might be like two to three is like free for whatever state they are?

**Therapist 41:55**

No, no, when it when it comes to the state counseling institution, then we could do how many we wanted?

**Interviewer 42:02**

That's interesting. All right. But of course, you also said what

**Therapist 42:07**

Yeah, once they told me there is like a specific number of sessions, I don't know, like 20 or 30. But nobody was really checking. So it was up to the workers up to the psychotherapists to decide how many,

**Interviewer 42:22**

And that was government subsidized clients didn't pay when that's amazing, or but you said that otherwise, it took around six to 10 sessions, then a three month break, and then they call you again, for like two to five sessions. And then there is some couples that, you know, like ongoing basis for you. It's already been a couple of years, for example. All right, 60 minutes, once every three weeks, but longer and longer as the therapy progresses. So you are also interested in the scale that we're developing, especially if there are norms and you know, a good psychometric property. But for you, for it to be useful to you either it's translated in [Country A]. If you're going to use it in session, you want it to be a short tool, but also you're also okay with it being a longer tool if you can develop prior to the session

thereafter. And we also talked about the price you prefer to be a monthly fee for were there a possibility for a longer but again, your it really depends on on how it's done the psychometric property, how its programmed user experience, etc. And you are interested in further collaborating for the development and scale. Alright, is there anything you'd like to add?

**Therapist 43:45**

No, no, that's about it.

**Interviewer 43:51**

All right. And before we end, I just like to talk to you about how we will handle the data. As a research project. Of course, you want this to be as open and as verifiable as possible, especially later on when people ask me about how we develop scale together. So there are two options. And let me well, at least two options and let me know which one you're more comfortable with. One option is that only portions of the transcript will be shared, none of which will be identified to you. Another option, if we were bolder, is that the whole transcript can be shared, removed of sensitive and overly identifying details. So it will be anonymized. However in this option, there is a minimal chance that someone who knows you professionally well can piece together the details and the transcript for example, your reality etc. and narrow it down and might have a suspicion that it is you have the two which one are you more comfortable with?

**Therapist 44:48**

The second one is fine. So you can you can share the transcript. It's not so interesting.

**Interviewer 44:56**

So I will I will get back to you with a transcript and if you want anything, if you want anything great back then we will read that.

**Therapist 45:07**

Yeah. It seems to me I wasn't really clear on some of the topics because it was the first time I was really thinking about them.

**Interviewer 45:17**

No, no, it was it was it was good. Very informative.